

Preventive Visit Billing Transition

During a preventive visit, discussion of new symptoms or the evaluation of existing medical problems may result in an additional Evaluation and Management (E/M) service being billed. When this occurs, the preventive service generally remains covered without cost-sharing under the Affordable Care Act (ACA), while the separately billed E/M service is subject to the patient's normal deductible, copayment, or coinsurance.

Understanding Modifier 25

Modifier 25 is a legitimate coding modifier defined by the American Medical Association (AMA) Current Procedural Terminology (CPT). It is intended to indicate that a physician or qualified healthcare professional performed a significant, separately identifiable E/M service on the same day as another service, such as a routine wellness check.

- **The Coding Mechanism:** The original preventive code (e.g., CPT 99396) remains intact and is processed at a \$0 member liability. If a distinct medical issue is evaluated, the provider appends a second E/M code (typically CPT 99212 through 99215) with Modifier 25.
- **Appropriate Application:** Improper application of Modifier 25 has been identified in audits conducted by Medicare contractors and commercial insurers. When used appropriately, however, Modifier 25 reflects legitimate additional physician work performed during the encounter.

Clinical Justification and Documentation

Whether Modifier 25 is appropriate depends on the provider's clinical documentation. Merely mentioning a stable chronic condition may not by itself justify a separately billable E/M service. Clinical documentation determines whether billing requirements have been met. To warrant a separate E/M charge, the medical record must demonstrate that the evaluation required independent history-taking, examination components, and medical decision-making (MDM) above and beyond the standard preventive protocol.

Financial Impact by Plan Tier

When an E/M code is added via Modifier 25, the resulting charge is applied directly to the enrollee's standard cost-sharing structures. The financial impact varies by health plan tier:

Plan Tier	Typical Cost-Sharing Impact
Bronze	High deductible exposure. The E/M charge is typically applied directly to the deductible, resulting in the patient paying the full negotiated rate out-of-pocket until the deductible is met.
Silver (No CSR)	Standard deductible exposure or fixed copayment, depending on the plan design.
Silver (With CSR)	Cost-sharing reduction (CSR) plans generally convert the charge to a fixed primary care or

Plan Tier	Typical Cost-Sharing Impact
	specialist copay.
Gold	Low deductible exposure or standard coinsurance/copay structures.

Resolution and Appeal Pathways

If a patient receives an Explanation of Benefits (EOB) indicating an unexpected E/M charge alongside a preventive visit, they can take the following administrative steps to review the claim:

1. **Request Documentation:** Patients can request their EOB, the carrier's medical policy, the clinical rationale, and the claim documentation (such as the CMS-1500 form) to verify how the code was applied and to confirm the presence of an independent diagnosis pointer.
2. **Internal Appeal:** The patient may file a formal internal appeal with their insurance carrier. Appeals should rely on ACA preventive service requirements, the specific insurance contract language, and the provider's clinical documentation.
3. **State Regulatory Review:** If the internal appeal is denied, Pennsylvania residents can file an administrative complaint with the Pennsylvania Insurance Department (PID) Bureau of Consumer Services.
4. **Act 146 External Review:** Under Pennsylvania Act 146, consumers have the right to request a state-administered independent external review for claim denials involving medical judgment, medical necessity, or appropriateness of service.
5. **Independent Dispute Resolution (IDR) Limits:** Patients cannot initiate the federal No Surprises Act Independent Dispute Resolution (IDR) process for Modifier 25 disputes. IDR is strictly an arbitration mechanism between providers, facilities, and insurers regarding payment rates. Consumers must rely on internal appeals, PID complaints, and Act 146 external reviews.